Constitutional Disparity, State Negligence, and the Exhaustion of Domestic Remedies: A Strategic Legal Framework for Challenging the Prohibition of Medical Cannabis in Ireland

1. Introduction: The Geopolitical and Constitutional Anomaly

The legal landscape regarding the regulation, prescription, and administration of cannabis-based products for medicinal use (CBPMs) in Western Europe has undergone a radical transformation in the last decade. However, the Republic of Ireland remains a legislative outlier, maintaining a prohibitionist framework that stands in stark contrast to the liberalization observed in the United Kingdom, Germany, and other peer jurisdictions. This report constitutes a comprehensive legal and evidential analysis designed to support a strategic litigation campaign. The objective is singular and procedurally precise: to initiate a constitutional challenge in the High Court of Ireland with the specific intent of eliciting a judicial dismissal. This dismissal is the requisite procedural key to exhausting domestic remedies, thereby unlocking the jurisdiction of the European Court of Human Rights (ECtHR) in Strasbourg.

The strategic necessity of this litigation arises from a unique intersection of legal standing and systemic state failure. The litigant possesses dual British and Irish citizenship, a status that confers rights under the Good Friday Agreement and the Common Travel Area (CTA). This dual status exposes a profound legal inequality: the litigant qualifies for medical cannabis access in the United Kingdom based on their National Insurance contribution record and clinical need, yet is criminalized for accessing the same treatment in Ireland due to residency. This geographic discrimination, contingent solely on which side of the Irish Sea the patient resides, arguably violates Article 14 (Prohibition of Discrimination) of the European Convention on Human Rights (ECHR) when taken in conjunction with Article 8 (Right to Respect for Private and Family Life).

Furthermore, this report integrates a forensic comparison of the Irish State's handling of the Sodium Valproate (Epilim) scandal against its current restriction of medical cannabis. We posit that the State's historical and ongoing negligence in permitting the prescription of a known teratogen (Valproate) to pregnant women—resulting in verifiable fetal mortality and severe

disability—shatters the credibility of its "public safety" defense regarding cannabis. The State cannot logically claim to restrict cannabis (a substance with zero recorded lethal overdose potential) on safety grounds while simultaneously countenancing the prescription of Valproate, which has killed and maimed thousands.

Finally, this analysis addresses the systemic violation of Article 3 (Prohibition of Torture and Inhuman or Degrading Treatment) within the Irish psychiatric system. We leverage reports from the European Committee for the Prevention of Torture (CPT) to demonstrate that the refusal to recognize Complex Post-Traumatic Stress Disorder (C-PTSD) under ICD-11 standards constitutes a form of medical abuse. By misdiagnosing trauma survivors with "mood disorders" and subjecting them to chemical restraint via inappropriate psychotropic medications, while denying access to cannabis-based therapies that regulate the endocannabinoid system, the State is complicit in the infliction of severe, avoidable suffering.

This document serves as the foundational "Book of Evidence" for the High Court proceedings, designed to force the State into a position where it must either concede the unconstitutionality of its position or, more likely, rely on a rigid separation of powers defense that creates the perfect test case for the European Court of Human Rights.

2. Comparative Regulatory Analysis: The Anglo-Irish Divergence

The chasm between the regulatory environments of Ireland and the United Kingdom is the primary engine of this litigation. Despite sharing a Common Law heritage, a Common Travel Area, and similar epidemiological profiles, the two jurisdictions have adopted diametrically opposing approaches to medical cannabis since 2018.

2.1 The United Kingdom: A Market-Led Liberalization

In November 2018, the UK Home Office rescheduled cannabis-based products for medicinal use (CBPMs) from Schedule 1 to Schedule 2 of the Misuse of Drugs Regulations 2001. This legislative maneuver fundamentally altered the clinical landscape by removing the political administration from the prescribing relationship.[1]

2.1.1 The Prescribing Mechanism

In the UK, the authority to prescribe lies with any specialist doctor listed on the General Medical Council (GMC) Specialist Register. Crucially, the legislation does not restrict prescribing to a defined list of conditions. A specialist may prescribe cannabis for *any* condition—be it chronic pain, PTSD, anxiety, or epilepsy—provided they are satisfied that there is clinical evidence of benefit and that licensed medicines have been exhausted or are unsuitable.[1] This clinical autonomy is the cornerstone of the UK model.

2.1.2 Scale of Access and Commercial Infrastructure

The liberalization of the law has catalyzed a robust private healthcare market. As of 2024/2025, there are over 300 specialized medical cannabis clinics operating across Great

Britain.[2] The patient cohort has expanded exponentially, with estimates placing the number of active legal patients at over 32,000 as of late 2022, with monthly growth rates suggesting a current figure significantly higher.[3]
The UK has also positioned itself as a dominant commercial hub in the European cannabis sector. It is the largest exporter of medical cannabis in Europe, primarily due to the production of Epidyolex and Sativex by GW Pharmaceuticals (now Jazz Pharmaceuticals), but increasingly through the licensing of high-THC cultivation for export.[5] This "exporter status" is legally significant; it demonstrates that the UK government recognizes the pharmaceutical legitimacy and economic value of the substance, rendering the Irish government's refusal to recognize its medicinal utility increasingly irrational.

2.1.3 The National Insurance Nexus

For the dual citizen, the UK system represents an accessible right. Possession of a National Insurance number allows for registration with UK private clinics (and in rare cases, NHS pathways). The eligibility for treatment is determined by clinical need and medical history, not by a Minister's political discretion. The user, by virtue of their British credentials, is a "qualified patient" in the eyes of the UK State.

2.2 Ireland: Restrictive, Bureaucratic, and Stagnant

In contrast, Ireland's approach is characterized by extreme caution, bureaucratic inertia, and a refusal to devolve power to clinicians. Cannabis remains a Schedule 1 substance under the Misuse of Drugs Acts, denoting a drug with "no medicinal use".[6] The exceptions to this prohibition are narrow and effectively non-functional for the majority of patients.

2.2.1 The Medical Cannabis Access Programme (MCAP)

The MCAP, launched in 2019 under SI 262/2019, was heralded as a compassionate access scheme. However, unlike the UK's open clinical discretion, the MCAP is statutorily bound to three specific conditions:

1. Spasticity associated with multiple sclerosis.
2. Intractable nausea and vomiting associated with chemotherapy.
3. Severe, refractory (treatment-resistant) epilepsy.[5]

This exhaustive list excludes chronic pain, which accounts for over 50% of medical cannabis prescriptions in the UK and Germany.[2] It also excludes psychiatric conditions such as PTSD, despite emerging evidence of efficacy. As a result, the programme has been a statistical failure. Five years after its inception, fewer than 60 patients have been funded or treated under the scheme.[7]

2.2.2 The Ministerial Licence: A Barrier to Care

For patients with conditions outside the MCAP (such as the user's potential chronic pain or PTSD), the only legal route is a Ministerial Licence. This requires a consultant to apply directly to the Minister for Health for a named-patient license. This process is described in legal and medical circles as "burdensome," "lengthy," and "bureaucratic".[5] It politicizes the

doctor-patient relationship, requiring executive approval for a medical decision. Furthermore, holding a Ministerial Licence does not guarantee state reimbursement; many patients are forced to pay thousands of euros monthly for products imported from the Netherlands (Bedrocan), creating a discriminatory wealth barrier to health.[5]

2.3 The Constitutional Disparity

The juxtaposition of these two regimes creates an intolerable legal anomaly for the dual citizen.

- **In the UK:** The patient is a legitimate medical user, accessing a Schedule 2 medicine prescribed by a specialist, contributing to the economy of a leading exporter.
- **In Ireland:** The same patient, with the same medical history and the same citizenship rights, is a criminal in possession of a Schedule 1 drug, or is forced to navigate a "pilot scheme" that explicitly excludes their condition.

This disparity is not merely a matter of differing policy choices; it is a breach of the **equality of treatment** implicitly guaranteed by the Common Travel Area and explicitly protected under EU non-discrimination principles (though complicated by Brexit) and the ECHR. The Irish State's refusal to recognize the validity of a UK prescription, or to offer an equivalent pathway, forces the citizen to choose between their health and their domicile.

Table 1: Regulatory Divergence – Ireland vs. United Kingdom (2024/2025)

Feature	Republic of Ireland (MCAP/Licence)	United Kingdom (Private/NHS)
Legal Classification	**Schedule 1** (Specified Controlled Drug)	**Schedule 2** (Medicinal Product)
Prescribing Authority	Restricted to Consultants; often requires Minister's sign-off.	Any Specialist Doctor on GMC Register.
Qualifying Conditions	**Strictly Limited (3):** MS Spasticity, Chemo Nausea, Severe Epilepsy.	**Unrestricted:** Clinician's discretion (Pain, PTSD, Anxiety, Neuro).
Patient Population	**~55** (approx.) [7]	**>32,000** (estimated) [3]
Infrastructure	No specialist clinics; Hospital-based only.	**300+** Specialist Clinics.[2]
Supply Chain	Net Importer (Reliance on Dutch Bedrocan).	Major Exporter; Domestic Cultivation.
Reimbursement	Restricted; Case-by-case (PCRS).	Private (mostly); Rare NHS funding.

---

3. The Sodium Valproate Scandal: Establishing State Hypocrisy and Negligence

To successfully challenge the constitutionality of the cannabis ban, the litigation must attack the State's primary defense: "Patient Safety." The State argues that cannabis is restricted because it is "unsafe" or "unproven." This defense collapses when contrasted with the State's permissive facilitation of Sodium Valproate (Epilim), a drug with a catastrophic safety profile.

3.1 The Teratogenic Catastrophe

Sodium Valproate is an effective anti-epileptic drug (AED). However, since the 1970s, it has been known to be highly teratogenic. Exposure in utero carries a **10% risk of major congenital malformations** (spina bifida, heart defects, limb deformities) and a **30-40% risk of serious neurodevelopmental disorders** (autism, low IQ, delayed walking/talking).[9]
In Ireland, it is estimated that **1,250 children** were born with Fetal Valproate Spectrum Disorder (FVSD) between 1975 and 2015.[11] These are not merely statistics; they represent a cohort of Irish citizens who were maimed by a State-licensed, State-reimbursed pharmaceutical product. The State failed to warn women of these risks for decades, despite the availability of data. This constitutes a systemic breach of the State's positive obligation to protect life and bodily integrity.

3.2 The "Survival" Argument: Mortality and Medical Cannabis

The user's query introduces a potent legal argument: that children died on Sodium Valproate who would have survived with medical cannabis. This refers to two distinct mortality vectors:

1. **Fetal/Neonatal Mortality:** Valproate exposure increases the risk of fetal death, stillbirth, and neonatal death due to severe malformations.[12] Cannabis, by contrast, has no known teratogenic lethality profile comparable to Valproate.
2. **Sudden Unexpected Death in Epilepsy (SUDEP):** The refusal to prescribe cannabis for severe epilepsy (Dravet/Lennox-Gastaut) forces patients to rely on drugs like Valproate or, in cases where Valproate fails, leaves them with uncontrolled seizures. Uncontrolled seizures are the primary cause of SUDEP. Evidence from the UK (e.g., the Zafar et al. study) demonstrates that whole-plant medical cannabis can reduce seizure frequency by **86-97%** in children who have failed on all other AEDs.[13]

Therefore, the State's restriction of cannabis is not a "safety" measure; it is a lethality multiplier. By denying access to a non-lethal anticonvulsant (cannabis) and steering patients toward a lethal teratogen (Valproate), the State has arguably caused preventable deaths.

3.3 The High Court Discovery Strategy

In the proposed High Court action, the legal team must seek **Orders for Discovery** requiring the State to produce:

- **Mortality Data:** Exact figures on neonatal deaths and stillbirths associated with maternal Valproate use from 1975 to present. The user demands the government

"disclose how many children have died." This is a justiciable demand.

- **Comparative Risk Assessments:** Any internal Department of Health memorandums comparing the safety profile of Valproate against Cannabidiol (CBD) or Tetrahydrocannabinol (THC). It is highly likely such assessments do not exist or are scientifically flawed, further proving the "irrationality" of the cannabis ban.
- **The Inquiry Records:** The Minister for Health has established a non-statutory inquiry into Valproate.[15] The litigation should seek to compel the immediate release of preliminary findings regarding "who knew what and when."

Table 2: The Safety Paradox – Valproate vs. Medical Cannabis

Metric	Sodium Valproate (Epilim)	Medical Cannabis (Whole Plant)
Legal Status in Ireland	**Licensed & Reimbursed**	**Schedule 1 (Prohibited/Restricted)**
Primary Indication	Epilepsy, Bipolar Disorder	Epilepsy, Chronic Pain, PTSD, MS
Pregnancy Risk	**Severe:** 10% Malformation, 40% Neuro-delay [10]	**Unknown/Low:** Caution advised, but no confirmed teratogenic syndrome.
Lethal Dose	**Yes:** Hepatotoxicity, Pancreatitis, Overdose risk.	**None:** Zero recorded fatal overdoses globally.
Seizure Efficacy	High (but with severe side effects)	High (in refractory cases) [13]
State Action	Distributed for 40+ years without adequate warning.	Blocked for 40+ years despite safety evidence.

---

4. Psychiatric Misdiagnosis and Article 3: The Torture Argument

The user identifies a critical failure in the Irish mental health system: the conflation of **Post-Traumatic Stress Disorder (PTSD)** and **Complex PTSD (C-PTSD)**, leading to "false diagnosis" and subsequent mistreatment. This section argues that such mistreatment reaches the threshold of Article 3 of the ECHR (Prohibition of Torture and Inhuman or Degrading Treatment).

4.1 The Diagnostic Schism: ICD-11 vs. DSM-5

The World Health Organization's **ICD-11** (International Classification of Diseases, 11th Revision) formally recognizes **Complex PTSD (C-PTSD)** as a distinct diagnosis. C-PTSD includes the core PTSD symptoms (re-experiencing, avoidance, threat sense) *plus*

"Disturbances in Self-Organization" (DSO):

1. Affect Dysregulation (inability to control emotions).
2. Negative Self-Concept (shame, guilt, feeling "worthless").
3. Interpersonal Difficulties (inability to sustain relationships).[16]

However, Irish psychiatric services frequently rely on the **DSM-5** (Diagnostic and Statistical Manual of Mental Disorders), which does *not* include C-PTSD as a standalone diagnosis. Consequently, patients presenting with DSO symptoms (affect dysregulation) are routinely misdiagnosed with **Borderline Personality Disorder (BPD)** or **Mood Disorders** (e.g., Bipolar Disorder).[18]

4.2 The "Chemical Restraint" as Inhuman Treatment

This misdiagnosis is not a benign clerical error; it dictates treatment.

- **Trauma Needs:** C-PTSD requires trauma-focused therapy (e.g., EMDR) and potentially symptom management via the endocannabinoid system (cannabis), which regulates fear extinction and memory.[21]
- **Mood Disorder Treatment:** Patients misdiagnosed with Bipolar/BPD are prescribed mood stabilizers (like Valproate/Lithium) and antipsychotics. These drugs do not treat the underlying trauma. Instead, they act as "chemical restraint," sedating the patient and suppressing emotion.[22]

Forcing a trauma survivor to ingest powerful psychotropic drugs that alter their consciousness without addressing their condition constitutes a violation of bodily integrity. When this occurs in a state institution (prison or psychiatric ward), and is combined with the denial of the *correct* treatment (cannabis/therapy), it meets the ECHR definition of **degrading treatment** (Article 3).

4.3 Evidence from the CPT (Committee for the Prevention of Torture)

The user's reference to "Strasburg European court of protection against torture" relates to the **CPT**, a body of the Council of Europe whose reports are heavily relied upon by the ECtHR. The CPT's reports on Ireland (2020-2025) provide damning evidence to support the High Court challenge:

1. **"Critically Deficient" Care:** The 2024 CPT report described mental healthcare in Irish prisons as "critically deficient".[23]
2. **Inappropriate Placement:** Prisons are being used to house severely mentally ill patients due to a lack of capacity in the Central Mental Hospital (CMH).[23]
3. **Degrading Practices:** The CPT highlighted the use of "observation cells" where vulnerable prisoners are stripped naked and left in "ponchos" without meaningful human contact. This practice was explicitly labeled as potentially "degrading".[24]
4. **Over-Medication:** The reports note a reliance on pharmacotherapy (drugs) over therapeutic activities, reinforcing the "chemical restraint" argument.[23]

The legal argument is straightforward: The State refuses to license medical cannabis (a viable treatment for PTSD symptoms) while simultaneously incarcerating mentally ill patients in conditions that violate international human rights standards and subjecting them to erroneous

medical regimes.

---

5. Procedural Strategy: Exhausting Domestic Remedies

The ultimate goal is to reach the European Court of Human Rights (ECtHR). However, the ECtHR is a court of last resort. Under **Article 35(1)** of the Convention, the Court "may only deal with the matter after all domestic remedies have been exhausted." This means the user cannot simply skip to Strasbourg; they must first bring a case in the Irish High Court and have it rejected.

5.1 The High Court Pleadings (The "Test Case")

The legal team must file for **Judicial Review** seeking:

1. **Certiorari:** To quash the decision of the Minister for Health to maintain Cannabis as Schedule 1.
2. **Declarations:**
    - That the MCAP's exclusion of C-PTSD/Chronic Pain is unconstitutional (irrational and disproportionate).
    - That the criminalization of a dual citizen for possessing medicine legal in their other jurisdiction violates Article 40.1 (Equality).
    - That the State has failed to vindicate the Right to Life (Article 40.3.2) by prioritizing Valproate over Cannabis.

5.2 The Necessary Dismissal

We anticipate, and indeed *require*, a dismissal. Irish courts have historically shown extreme deference to the Executive (Government) in matters of health policy and resource allocation (*Ryan v. Attorney General*, *T.D. v. Minister for Education*). The State will argue:

- **Separation of Powers:** "It is for the Minister and the Oireachtas to schedule drugs, not the Courts."
- **Scientific Uncertainty:** "The HPRA advises that evidence is insufficient."

The High Court will likely accept these arguments and dismiss the case, stating that it cannot usurp the Minister's function. **This judgment is the "Golden Ticket."** It proves to the ECtHR that there is *no effective remedy* available in Irish law. The user has tried, and the Irish system has declared itself powerless to help.

5.3 Appeal to the Supreme Court

To be absolutely safe on "exhaustion," the dismissal should be appealed to the Court of Appeal or Supreme Court. Once the final appellate court dismisses the case, the 4-month clock for the ECtHR application begins.

---

6. The European Court of Human Rights (ECtHR) Application

With domestic remedies exhausted, the application to Strasbourg is lodged. The arguments here shift from Irish Constitutional Law to the ECHR Articles.

6.1 Article 14 (Discrimination) combined with Article 8

- **The Argument:** The applicant is subject to discrimination in the enjoyment of their private life (medical autonomy) based on "residence" and "legal status."
- **The Comparator:** A British citizen resident in Belfast (UK) with the same condition can access this medicine. The applicant, a British citizen resident in Dublin, cannot. Within the Common Travel Area, which purports to offer reciprocal rights, this difference in treatment is devoid of "objective and reasonable justification."
- **European Consensus:** The ECtHR often looks to the "consensus" among member states. With the UK, Germany, Malta, and others liberalizing access, Ireland is an outlier. The Court may find Ireland's "margin of appreciation" (discretion) has been exceeded because the restriction is no longer "necessary in a democratic society."

6.2 Article 3 (Inhuman and Degrading Treatment)

- **The Argument:** The State's refusal to allow access to the *only* effective treatment (cannabis) for the applicant's condition (be it seizures or trauma), while offering only toxic alternatives (Valproate) or ineffective ones, subjects the applicant to "intense physical and mental suffering."
- **The *Paposhvili* Standard:** In *Paposhvili v. Belgium* [25], the Grand Chamber expanded the scope of Article 3 in medical cases. It ruled that a violation occurs if a person is exposed to a "serious, rapid and irreversible decline in his or her state of health resulting in intense suffering."
- **Application:** The user argues that the denial of cannabis leads to unremitting seizures (risk of death/brain damage) or severe psychological trauma (C-PTSD), constituting "intense suffering" that the State has the power to prevent but chooses not to.

6.3 Article 2 (Right to Life)

- **The Argument:** By restricting a non-lethal medicine that prevents seizures (Cannabis) and permitting a lethal one (Valproate), the State has failed its positive obligation to protect life. The statistical probability of death (SUDEP) is higher under the current Irish regime than it would be under the UK regime.

---

7. Conclusion

The disparity between the medical cannabis regimes of Ireland and the United Kingdom is not

merely an administrative difference; it is a human rights violation manifest in law. For the dual citizen, it represents a denial of identity and a threat to life.
By challenging this regime, the applicant exposes the Irish State's hypocrisy: a State that claims to restrict cannabis for "safety" while presiding over the Sodium Valproate tragedy; a State that claims to treat mental illness while the CPT reports it is warehousing patients in degrading conditions; a State that claims to honor the Common Travel Area while stripping citizens of their cross-border health rights.
The path forward is clear:

1. **Initiate High Court Judicial Review** immediately, citing the Valproate inquiry and CPT reports as evidence of systemic state failure.
2. **Secure the Dismissal** on separation of powers grounds to satisfy Article 35 ECHR.
3. **Lodge the Application in Strasbourg**, framing the case as a unique violation of Articles 3, 8, and 14 involving a dual citizen within the CTA.

This litigation has the potential not only to vindicate the rights of the individual user but to force a fundamental realignment of Irish drug policy with European human rights standards.

Table 3: The Path to Strasbourg – Procedural Steps

Stage	Action	Objective	Legal Basis
1. Discovery	Request Valproate mortality data & CPT response records.	Prove State negligence & lack of "safety" logic.	*Rules of the Superior Courts*
2. High Court	Judicial Review of MCAP restrictions.	Challenge Constitutionality (Bodily Integrity).	*Bunreacht na hÉireann* Art 40.3
3. Judgment	**DISMISSAL EXPECTED.**	Establish "No Effective Remedy" exists.	Separation of Powers Doctrine
4. Appeal	Appeal to Supreme Court/Court of Appeal.	Final Exhaustion of Remedies.	*Article 35(1) ECHR*
5. ECtHR	File Application to Strasbourg.	Claim violation of Convention Rights.	*Articles 2, 3, 8, 14 ECHR*

Citations

[1]

Works cited

1. Medical use of cannabis - The House of Commons Library, accessed February 11, 2026, https://commonslibrary.parliament.uk/research-briefings/cbp-8355/
2. New UK Report Highlights Untapped Cannabis Industry Potential, accessed February 11, 2026,

https://internationalcbc.com/new-uk-report-highlights-untapped-cannabis-industry-potential/
3. Neurologists' perspectives of cannabis-based medicines: results from an all-Ireland survey, accessed February 11, 2026, https://pmc.ncbi.nlm.nih.gov/articles/PMC12031982/
4. Medical cannabis prescriptions - the facts behind the headlines | News, accessed February 11, 2026, https://www.ukmccs.org/medical-cannabis-prescriptions-the-facts-behind-the-headlines/
5. Medical Cannabis in Ireland 2025 - Prohibition Partners, accessed February 11, 2026, https://prohibitionpartners.com/2025/09/09/medical-cannabis-in-ireland-access-regulation-and-patient-pathways-2025/
6. Cannabis for Medical Use Information for Patients and healthcare professionals., accessed February 11, 2026, https://www.drugsandalcohol.ie/29336/1/Medicinal-Cannabis-Document-for-Publication.pdf
7. Dáil Éireann debate. Questions on Policy and Legislation [medical cannabis access programme]., accessed February 11, 2026, https://www.drugsandalcohol.ie/40408/
8. Medical Cannabis Products Reimbursement Scheme - HSE, accessed February 11, 2026, https://www2.hse.ie/services/schemes-allowances/medical-cannabis-products-reimbursement-scheme/
9. More than 1,000 Irish children potentially affected by valproate syndrome, estimates HSE study - Medical Independent, accessed February 11, 2026, https://www.medicalindependent.ie/in-the-news/news-features/more-than-1000-irish-children-potentially-affected-by-valproate-syndrome-estimates-hse-study/
10. AN COMHCHOISTE UM SLÁINTE Tuarascáil maidir le ... - Oireachtas, accessed February 11, 2026, https://data.oireachtas.ie/ie/oireachtas/committee/dail/32/joint_committee_on_health/reports/2018/2018-06-05_report-on-foetal-anti-convulsant-syndrome_en.pdf
11. Health Services Reports – Wednesday, 15 May 2019 - Oireachtas, accessed February 11, 2026, https://www.oireachtas.ie/en/debates/question/2019-05-15/158/
12. Sodium Valproate: The Fetal Valproate Syndrome Tragedy | Harm ..., accessed February 11, 2026, https://university.open.ac.uk/research-centres/herc/blog/sodium-valproate-fetal-valproate-syndrome-tragedy
13. Medical cannabis for children with hard-to-treat epilepsy, accessed February 11, 2026, https://www.epilepsy.org.uk/news/whole-plant-medical-cannabis-effective-and-well-tolerated-in-children-with-hard-to-treat-epilepsy

14. Medical cannabis for severe treatment resistant epilepsy in children: a case-series of 10 patients - PMC, accessed February 11, 2026, https://pmc.ncbi.nlm.nih.gov/articles/PMC8689159/
15. Minister for Health welcomes the commencement of Sodium Valproate Inquiry, accessed February 11, 2026, https://www.gov.ie/en/department-of-health/press-releases/minister-for-health-welcomes-the-commencement-of-sodium-valproate-inquiry/
16. Does Requiring Trauma Exposure Affect Rates of ICD-11 PTSD and Complex PTSD? Implications for DSM–5 | Psychological, accessed February 11, 2026, https://www.ptsd.va.gov/professional/articles/article-pdf/id1558737.pdf
17. ICD-11 complex post-traumatic stress disorder: simplifying diagnosis in trauma populations | The British Journal of Psychiatry - Cambridge University Press, accessed February 11, 2026, https://www.cambridge.org/core/journals/the-british-journal-of-psychiatry/article/icd11-complex-posttraumatic-stress-disorder-simplifying-diagnosis-in-trauma-populations/E53B8CD7CF9B725FE651720EE58E93A4
18. Trauma exposure and co-occurring ICD-11 post-traumatic stress disorder and complex post-traumatic stress disorder in adults with lived experience of psychiatric disorder - PMC, accessed February 11, 2026, https://pmc.ncbi.nlm.nih.gov/articles/PMC9543812/
19. CPTSD Misdiagnosis: 14 Diagnoses That Might Really Be CPTSD - Empower Counseling, accessed February 11, 2026, https://empowercounseling.net/blog/cptsd-misdiagnosis/
20. The Misdiagnosis and Ignorance of Complex PTSD | CPTSDfoundation.org, accessed February 11, 2026, https://cptsdfoundation.org/2023/12/27/the-misdiagnosis-and-ignorance-of-complex-ptsd/
21. PTSD | UK Medical Cannabis Registry - Curaleaf Clinic, accessed February 11, 2026, https://curaleafclinic.com/medical-cannabis-and-post-traumatic-stress-disorder-insights-from-the-uk-medical-cannabis-registry/
22. Inside Out Issue 101 Article 10 - IAHIP, accessed February 11, 2026, https://iahip.org/InsideOut-101-Article10
23. Council of Europe anti-torture committee (CPT) publishes report on Ireland, highlighting serious concerns about prisons, accessed February 11, 2026, https://www.coe.int/en/web/cpt/-/council-of-europe-anti-torture-committee-cpt-publishes-report-on-its-2024-periodic-visit-to-ireland
24. CPT Report 2020: Summary of Critical Issues | Irish Penal Reform Trust, accessed February 11, 2026, https://www.iprt.ie/latest-news/cpt-report-2020-summary-of-critical-issues/
25. Moving towards Compliance ? Medical Expulsion in Ireland in Light of Paposhvili - University of Leicester research repository - Figshare, accessed February 11, 2026, https://figshare.le.ac.uk/articles/journal_contribution/Moving_towards_Alignment_with_Strasbourg_Medical_Expulsion_in_Ireland_in_Light_of_Paposhvili/19350470/1

/files/34363565.pdf
26. Medical Cannabis Access Program - MCAP - O'Kelly Solicitors, accessed February 11, 2026, https://www.okellysolicitors.ie/2023/medical-cannabis-in-ireland/
27. Minister Donnelly must honour commitment for valproate inquiry - Rose Conway Walsh TD, accessed February 11, 2026, https://sinnfein.ie/news/minister-donnelly-must-honour-commitment-for-valproate-inquiry-rose-conway-walsh-td/
28. The CPT and Ireland - The Council of Europe, accessed February 11, 2026, https://www.coe.int/en/web/cpt/ireland
29. Clinical Outcome Data of Children Treated with Cannabis-Based Medicinal Products for Treatment Resistant Epilepsy—Analysis from the UK Medical Cannabis Registry - PubMed Central, accessed February 11, 2026, https://pmc.ncbi.nlm.nih.gov/articles/PMC10166640/
30. Diagnosis and management of complex post-traumatic stress disorder (C-PTSD) | London Waiting Room, accessed February 11, 2026, https://londonwaitingroom.nhs.uk/sites/default/files/2025-06/CPTSD%20in%20BMJ-2024-079458.full_.pdf
31. Complex post-traumatic stress disorder: Diagnosis and treatment ..., accessed February 11, 2026, https://www.medicalindependent.ie/clinical-news/complex-post-traumatic-stress-disorder-diagnosis-and-treatment/
32. Common Travel Area: rights of UK and Irish citizens - GOV.UK, accessed February 11, 2026, https://www.gov.uk/government/publications/common-travel-area-guidance
33. Cross border services and reciprocal healthcare FAQs | Department of Health, accessed February 11, 2026, https://www.health-ni.gov.uk/articles/cross-border-services-and-reciprocal-healthcare-faqs
34. Ending the pain of children with severe epilepsy? An audit of the impact of medical cannabis in 10 patients - Bedrocan, accessed February 11, 2026, https://bedrocan.com/wp-content/uploads/ending-the-pain-of-children-with-severe-epilepsy_zafar_2020.pdf
35. Medical Cannabis Access Programme (MCAP) - HPRA, accessed February 11, 2026, https://www.hpra.ie/regulation/controlled-drugs/medical-cannabis-access-programme
36. Medical Cannabis Access Programme, accessed February 11, 2026, https://www.gov.ie/en/department-of-health/publications/medical-cannabis-access-programme/